



ChatHealthy.ai

Find Care. Evaluate Care. Discuss Care.

Business Architecture

ChatHealthy.ai provides the data, tools, and an AI-enlightened space to empower regular people to prevail over the broken healthcare system. Whatever a person's circumstances might be, they deserve the best possible care, at the best value. ChatHealthy.ai gives them the platform to get it. By presenting and evaluating care options but never giving medical advice, the platform allows families, individuals, and friends to collaborate with the most powerful AI models in order to choose the best provider they can have.

Copyright 2026 Skip Snow. All rights reserved.

Executive Summary

Healthcare is the largest consumer market in the world, yet the individual remains the least empowered participant. Decisions are fragmented across search, opaque quality data, and isolated conversations with no coordination. The result is predictable: overpayment, underperformance, and preventable harm. ChatHealthy.ai exists because this gap is not a feature of the system -- it is the system's failure point.

ChatHealthy.ai unifies the healthcare decision process into a single consumer-controlled environment: find care, evaluate it with real quality signals, and decide collaboratively with people and AI. This is not another point solution. It is the decision layer sitting above the entire healthcare system, where choices are actually made. Once this layer exists, control shifts from institutions to individuals.

The business model follows the control point. We own the decision room. AI companies, not patients, pay to participate in it. As healthcare decisions become AI-assisted and collaborative by default, ChatHealthy.ai becomes the gateway through which those decisions flow. This is not a feature set. It is inevitable infrastructure.

Consumer Orchestrates the Care Journey

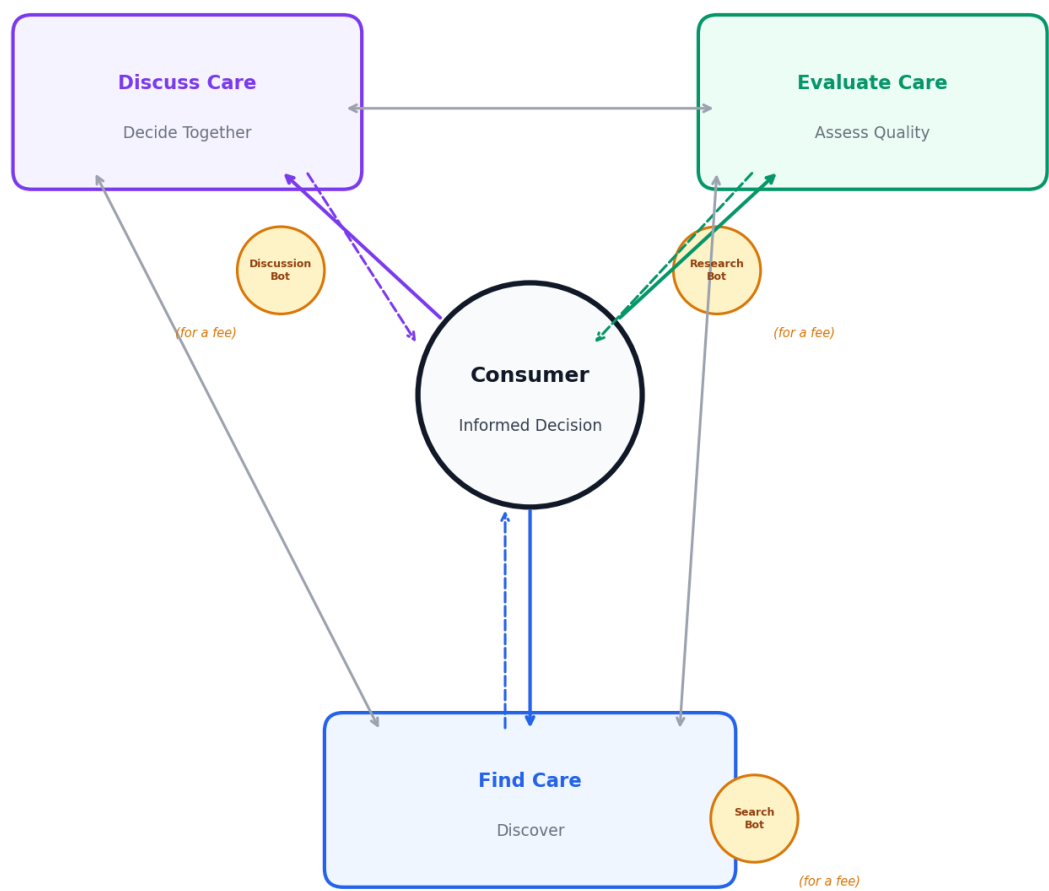


Fig. 1: The consumer orchestrates the care journey. Three components serve the decision, not a linear pipeline.

Platform Architecture

ChatHealthy.ai is built on three business components. The consumer is at the center, orchestrating the relationship between all three. This is not a sequential journey -- it is a decision-making environment.

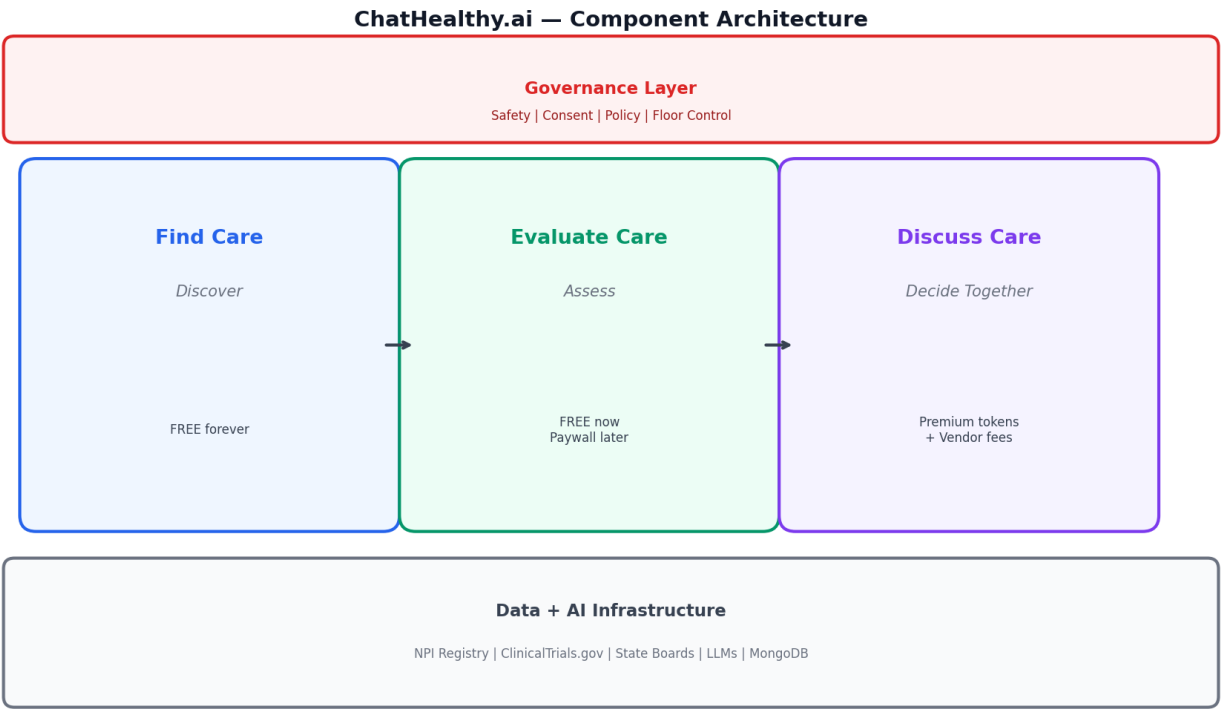


Fig. 2: Three components share a governance layer and data infrastructure. Each operates independently but communicates through defined interfaces.

Find Care

The entry point to the healthcare system, redesigned for clarity and control. Users discover providers, specialties, and care pathways without friction or bias.

- Provider and specialty discovery
- Navigation of care pathways
- Geographic and access-based filtering
- Integrated entry into evaluation and discussion

Pricing: Free forever

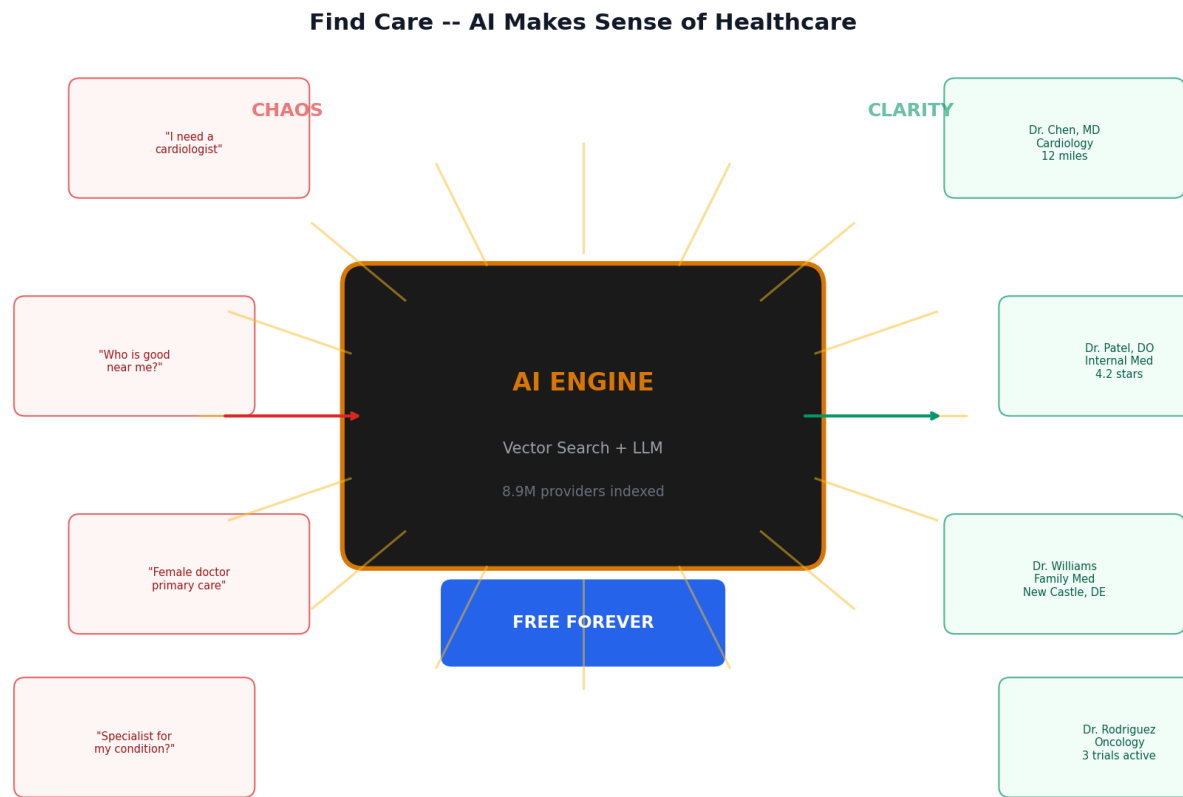


Fig. 3: Chaos in, clarity out. The AI engine indexes 8.9M providers and translates natural language into precise matches. Free forever.

Evaluate Care

Transforms fragmented public and clinical data into clear, consumer-comprehensible quality signals using proprietary AI synthesis.

- Clinical trials discovery and matching
- Provider credential analysis
- Quality signal aggregation and interpretation
- Comparative provider evaluation

Pricing: Free initially, with premium paid tiers for advanced insights and deeper analysis

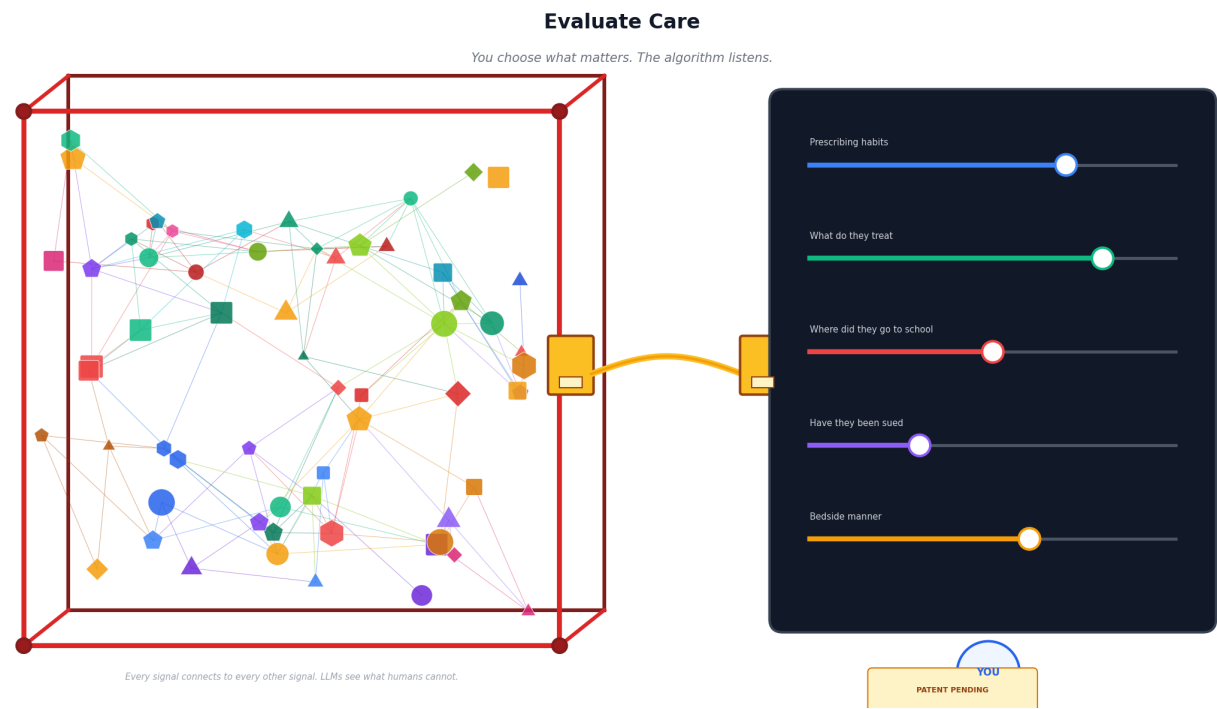


Fig. 4: Public data flows into a mixing board. The consumer tunes the algorithm. The output is a personalized provider ranking. Patent pending.

Discuss Care

A real-time, multi-participant decision environment where individuals bring family, caregivers, and multiple AI models into a single structured conversation.

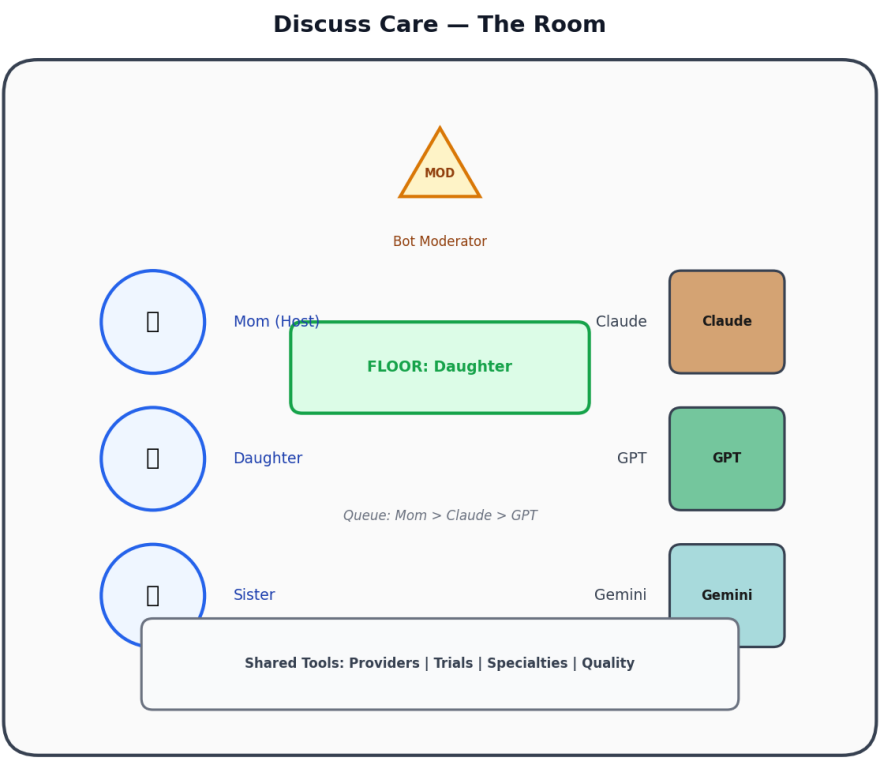


Fig. 5: Inside a Discuss Care session: humans and AI models collaborate with shared tools. A bot moderator manages the floor.

Features (12)

- DC-01: Shared Chat Session -- Multiple humans in one conversation with AI assistance
- DC-02: Invite System -- User sends invite link to friends, family, caregivers. No account required.
- DC-03: Multi-Model AI -- Multiple AI models participate in the same session (Claude, GPT, etc.), each with access to all tools
- DC-04: Shared Tool Access -- All participants (human and AI) can trigger provider search, clinical trials, specialty identification, provider detail
- DC-05: Healthcare Guardrails -- Conversations constrained to healthcare navigation. Same safety filter, emergency detection, scope rules.
- DC-06: Session Transcript -- Exportable transcript for sharing with doctors, care teams
- DC-07: Consent per Participant -- Each participant manages their own consent tier independently
- DC-08: Participant Roles -- Session creator (host), invited participant (peer). No admin/enterprise roles.
- DC-09: Real-Time Collaboration -- All participants see messages and AI responses live
- DC-10: Session Persistence -- Sessions survive disconnects, resumable by invite link
- DC-14: Shared Research Bot -- Any participant or component can dispatch a research bot to gather information. Shared object across all three components and pip
- DC-15: Licensed Provider Detection and Compliance Disclosure -- If a participant identifies as or is detected as a licensed healthcare provider, the system triggers

Pricing: Premium via AI token usage markup; users pay only when they choose advanced AI participation

Revenue Model

- AI token markup on premium model usage within Discuss Care
- AI vendor access fees to participate in decision sessions
- Future premium subscriptions for advanced evaluation insights

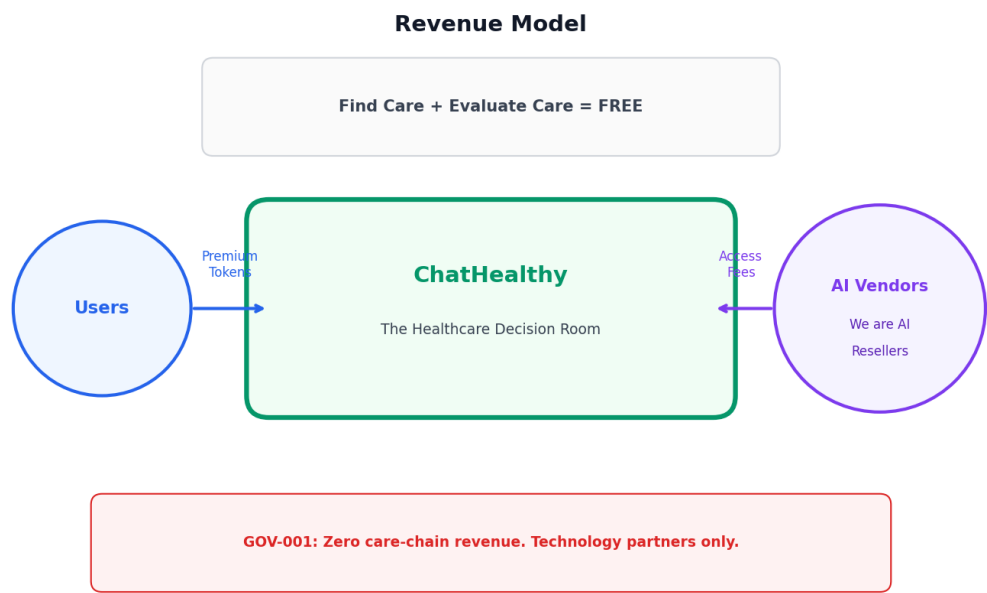


Fig. 6: Two-sided platform: consumers pay token premiums, AI vendors contribute capital. Core services remain free.

Competitive Moat

Regulatory Willingness

AI vendors avoid FDA classification and HIPAA compliance. They need someone else to own the healthcare room.

Governance Framework

Multi-AI safety, consent, floor control, moderation. Nobody else has this.

Data + Network

Provider, trial, and quality signals compound over time. Takes years to build.

Consumer Trust

Years to build, seconds to lose. Trust is the ultimate barrier.

Patent

LLM-based synthesis of provider quality signals. Method patent pending.

IP Protection & Trade Secrets

Proprietary algorithms, data processing methods, and governance frameworks.

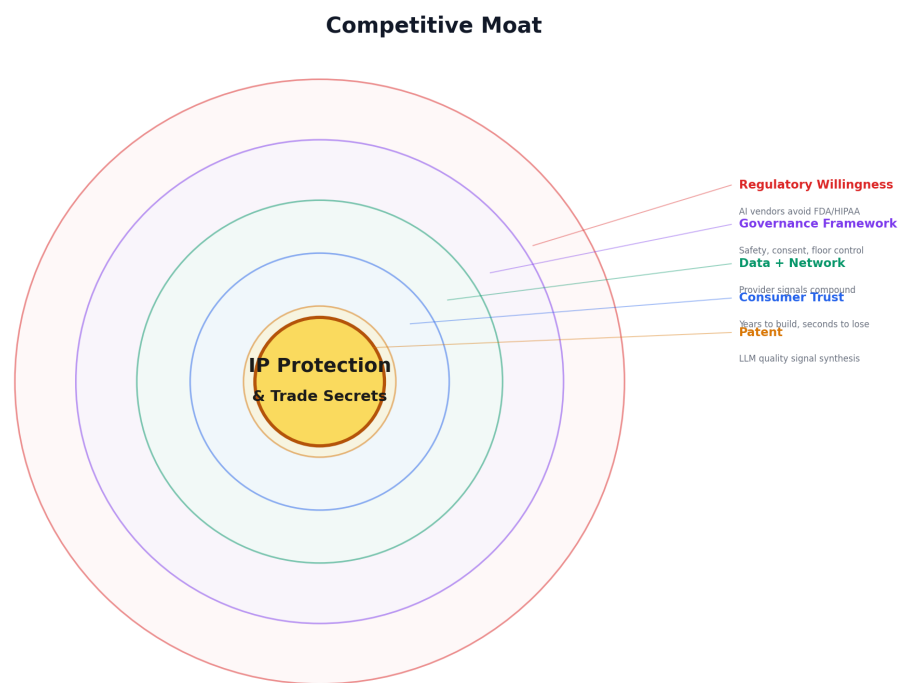


Fig. 7: Five concentric barriers protect the platform. Trade secrets and IP at the core.

Security Positioning

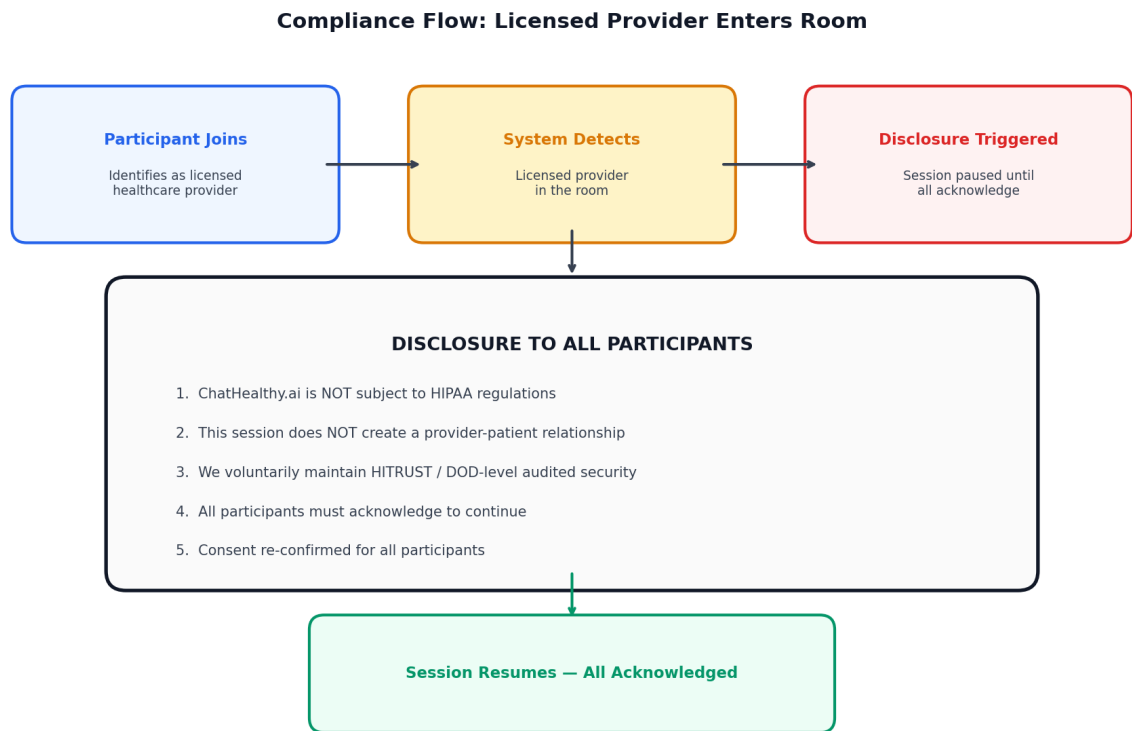


Fig. 8: When a licensed provider enters the room, the system pauses for mandatory disclosure. All participants must acknowledge.

Investment Thesis

ChatHealthy.ai sits at the intersection of two inevitable shifts: consumer control of healthcare decisions and AI participation in those decisions. The company captures value by owning the environment where both converge. Investors are backing a platform that becomes the default interface for navigating a \$4 trillion market. Strategic value is immediate for AI companies seeking distribution, and long-term for any entity that benefits from influencing healthcare decisions without regulatory exposure.

Timeline

2026-09-30 -- iPhone App Launch

Competitive Landscape

The healthcare navigation market is dominated by platforms that earn revenue from the providers they claim to evaluate. ChatHealthy.ai is the only platform built entirely on the consumer side of the table.

Every incumbent earns money from the care chain -- providers, hospitals, or payers. This creates a structural conflict of interest. The consumer cannot trust a platform that is paid by the people it is supposed to evaluate.

Yelp Health

Earns revenue from sponsored listings. Providers who pay appear first. Consumer trust is the product being sold.

ZocDoc

Charges providers for patient bookings. The platform is a referral engine disguised as a search tool.

Healthgrades

Revenue from provider advertising and hospital partnerships. Ratings influenced by who pays.

Angies List

Merged with HomeAdvisor. Lead generation model. Providers pay for leads. Consumer is the product.



Fig. 9: The healthcare navigation market. ChatHealthy.ai is the only platform that takes zero revenue from the care chain.

ChatHealthy.ai will never take money from providers, hospitals, payers, pharmaceutical companies, PBMs, or any entity in the care delivery or payment chain. This is not a guideline. It is governance policy GOV-001, permanent and non-negotiable. Our revenue comes exclusively from consumers (token premiums) and technology partners (AI vendor capital). This is our moat. No incumbent can match it without destroying their business model.